

MediGuard Health Plan — Prior Authorization Guidelines

Utilization Management Department | Updated March 2024

The following procedures require prior authorization before services are rendered. Failure to obtain prior authorization may result in a reduction or denial of benefits. Urgent and emergent services are exempt from prior authorization requirements. Authorization decisions are made within 5 business days for standard requests and 72 hours for urgent requests.

CPT 27447 — Total Knee Replacement (Total Knee Arthroplasty)

Clinical Criteria for Authorization:

1. Patient must be 50 years of age or older, or have documented post-traumatic arthritis.
2. Documentation of at least 6 months of conservative treatment failure, including physical therapy (minimum 12 sessions), NSAIDs or analgesic medication trial, and at least one corticosteroid injection.
3. Radiographic evidence of moderate to severe osteoarthritis (Kellgren-Lawrence grade III or IV).
4. Functional impairment documented by validated outcome measure (e.g., WOMAC score, KOOS).
5. BMI must be below 45 kg/m². Patients with BMI 40-45 require additional surgical risk assessment.

Required Documentation: Orthopedic consultation notes, imaging reports (X-ray within 6 months, MRI if available), physical therapy records, medication history, BMI documentation.

Review Turnaround: Standard: 5 business days. Peer-to-peer review available within 48 hours of adverse determination.

Authorization Validity: Authorization valid for 90 days from date of approval.

CPT 70553 — Brain MRI with and without Contrast

Clinical Criteria for Authorization:

1. Clinical justification required: new onset neurological symptoms (seizures, focal deficits, severe headache with red flags), suspected intracranial mass or lesion, follow-up of known intracranial pathology, or pre-surgical planning.
2. Prior non-contrast CT or initial MRI must be documented if request is for follow-up imaging.
3. Contrast component must be justified: suspected tumor, infection, demyelinating disease, or vascular malformation.
4. Routine headache without red flags does not meet criteria for contrast-enhanced MRI.

Required Documentation: Ordering physician's clinical notes, prior imaging reports if applicable, neurological examination findings.

Review Turnaround: Standard: 3 business days. Expedited review for suspected malignancy: 24 hours.

Authorization Validity: Authorization valid for 60 days from date of approval.

CPT 29881 — Knee Arthroscopy with Meniscectomy

Clinical Criteria for Authorization:

1. MRI results must confirm meniscal tear (must be submitted with authorization request).
2. Patient must have failed at least 6 weeks of conservative management including rest, ice, compression, physical therapy, and anti-inflammatory medication.
3. Mechanical symptoms (locking, catching, giving way) must be documented.
4. For patients over 50 with concomitant osteoarthritis, authorization requires documentation that the mechanical symptoms are primarily attributable to the meniscal tear.

Required Documentation: MRI report with radiologist interpretation, orthopedic evaluation notes, physical therapy records, documentation of conservative treatment timeline.

Review Turnaround: Standard: 5 business days.

Authorization Validity: Authorization valid for 60 days from date of approval.

Appeal Process

If a prior authorization request is denied, the requesting provider or member may submit an appeal within 180 days of the adverse determination. First-level appeals are reviewed by a physician reviewer who was not involved in the initial decision. Second-level appeals are reviewed by an external independent review organization (IRO).